

Opportunistic Salpingectomy for Prevention of Tubo-Ovarian Carcinoma

The European Society of Gynaecological Oncology Consensus Statements

Jurgen M. Piek, MD, PhD; Jolijn Schauwaert, MD; Laura Burney Ellis, MBChB; Ignacio Zapardiel, MD, PhD; François Planchamp, MSc; Kata Koblos; Joanna Kacperczyk-Bartnik, MD, PhD; Sarah J. Bowden, MBBS, PhD; Houssein El Hajj, MD, PhD; Mihaela Grigore, MD, PhD; Miranda P. Steenbeek, MD, PhD; Nicolò Bizzarri, MD, PhD; Maria Kyrgiou, MD, PhD; Murat Gültekin, MD

IMPORTANCE The fallopian tube epithelium has been demonstrated to be an important source of tubo-ovarian carcinoma. Therefore, removal of the fallopian tubes during unrelated pelvic or abdominal surgery (opportunistic salpingectomy) can potentially lower future ovarian cancer risk.

OBJECTIVES To assess current evidence on the efficacy, risks, and long-term outcomes of opportunistic salpingectomy and to develop consensus statements for the European Society of Gynaecological Oncology.

EVIDENCE REVIEW An international working group of 14 individuals including a patient representative was formed to develop consensus statements on opportunistic salpingectomy. The MEDLINE database was used to conduct a literature review of English-language studies from January 1, 2000, through March 1, 2025, evaluating opportunistic salpingectomy for reduction of tubo-ovarian carcinoma, complication rates, additional surgical time, and impact on ovarian function. Statements were subsequently drafted collaboratively based on the review of the literature and adapted in an iterative process in conference call meetings with opportunity for anonymous and nonanonymous feedback. The anonymous voting was binary (agree/disagree) for each potential statement. Final statements reached consensus with more than 75% agreement.

FINDINGS In the literature review, 230 studies were identified, of which 129 were deemed relevant to consensus statement development. Consensus was achieved on 18 statements, with grades of recommendation ranging from B to D and levels of evidence from II to V. Opportunistic salpingectomy is significantly associated with a lower risk of subsequent tubo-ovarian carcinoma, with no adverse short-term impact on ovarian function. The procedure appears safe across surgical approaches, with little additional operative time. Existing evidence does not indicate harm to ovarian function or premature menopause, although long-term evidence is not available. Salpingectomy is feasible during both gynecological and nongynecological procedures and should be considered in women undergoing gynecological surgery and, where possible, in women undergoing selected nongynecological pelvic or abdominal surgeries.

CONCLUSIONS AND RELEVANCE Existing evidence demonstrates that opportunistic salpingectomy is significantly associated with a lower risk of developing tubo-ovarian carcinoma. Clinicians should include this prevention intervention in preoperative counseling of eligible women.

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Author Affiliations: Author affiliations are listed at the end of this article.

Corresponding Author: Maria Kyrgiou, MD, PhD, Institute of Reproductive and Developmental Biology, Imperial College London, Du Cane Road, London W12 0NN, United Kingdom (m.kyrgiou@imperial.ac.uk).

Ovarian cancer remains one of the most lethal gynecological malignancies,¹ with negligible improvement in the 5-year survival rate over the past decades despite advancements in treatment. Most cases are diagnosed at an advanced stage due to a lack of effective screening strategies. Efforts to reduce death from this cancer now include risk-reducing surgical prevention interventions.

Emerging evidence has implicated the fallopian tube as the site of origin for a significant amount of high-grade serous ovarian cancers, the most common form of epithelial ovarian cancer, thus referred to as *tubo-ovarian carcinoma*.² This paradigm change led to the development and adoption of opportunistic salpingectomy, the removal of the fallopian tubes at the time of other indicated surgical procedures, as a preventive approach.³ Although initially proposed for high-risk populations, such as *BRCA1* and *BRCA2* gene pathologic variant carriers, opportunistic salpingectomy is now being implemented more broadly in women with average risk who are undergoing pelvic and selected abdominal surgeries for other indications.

Opportunistic salpingectomy has growing support from professional societies and public health groups worldwide. Yet despite its potential to reduce tubo-ovarian carcinoma incidence without apparent significant additional risk, in practice this has not been widely incorporated into patient care, with variability across specialties, institutions, and countries.^{4,5} This variation reflects ongoing debates regarding cost-effectiveness, surgical safety, and long-term outcomes.

This Special Communication aims to provide a comprehensive overview of the rationale for opportunistic salpingectomy in women who do not wish to retain fertility, summarizing the evidence supporting its implementation and exploring factors influencing its adoption into clinical practice, and ultimately to provide evidence-based consensus statements.

Methods

Ethics

This research was based on analysis of publicly available or previously published data and was therefore exempt from institutional review.

The European Society of Gynaecological Oncology (ESGO) consensus statements on opportunistic salpingectomy were developed using an 8-step process (Figure).

Selection of the Panel

Members of ESGO include gynecologists, gynecological oncologists, medical and clinical oncologists, radiation oncologists, and specialized nurses. The ESGO Prevention Committee consists of field experts and their research teams, selected from among the ESGO members for their specialism in research or campaigns specifically in the prevention of gynecological cancers, with professional diversity and geographical representation across Europe.⁶ ESGO Prevention Committee members and external experts with relevant expertise were nominated by the Prevention Committee executive council to develop this consensus statement. The ESGO Executive Council validated the final roster of specialists who were selected from the nomination short list from 9 different European countries, based on

subject matter expertise, number of relevant publications, and previous experience with development of consensus guidelines. The final group of specialists were well-recognized experts who were engaged in clinical and research activity with demonstrated leadership in the field; members of their research teams were included. Specialists were asked to base their decisions on their own experience and expertise. The final working group included 7 gynecological oncologists, 1 obstetrician-gynecologist, 2 gynecological oncology fellows, 2 gynecology residents, 1 methodologist (F.P.), and 1 patient representative. The methodologist provided the methodological support for the entire process and did not participate in voting for the consensus statements.

Identification of the Evidence

Prior to the first group meeting, a literature review was conducted based on 10 distinct PICO frameworks (PICO stands for patient or problem; intervention or exposure; comparison or control; outcome[s]). The PICO frameworks are listed in the eAppendix in the Supplement. The MEDLINE database was used to identify studies published in English from January 1, 2000, through March 1, 2025, by a methodologist (F.P.); the full search strategy is listed in the eAppendix in the Supplement. The evidence was assessed for quality and assigned a level of evidence (LOE), with emphasis on randomized clinical trials (RCTs), systematic reviews, and meta-analyses. Studies with lower LOEs, such as prospective cohort studies and retrospective registry-based analyses, were also evaluated. Editorials, case report, letters, in vitro studies, and publications in languages other than English were excluded. The reference list of each identified article was reviewed for other potentially relevant articles. The final results of the literature search, with electronic full-text versions of each article, were distributed to the whole group (eFigure in the Supplement). The full group was subdivided into working groups per PICO. All identified articles were reviewed by the working groups for relevance to their respective PICOs, in which studies were excluded if they focused on women at high risk instead of the general population. All articles relevant to the PICOs are listed in the eReferences in the Supplement.

Process

The group used a modified Delphi technique to develop and refine statements.⁷ At an initial conference call meeting led by J.M.P., the group was presented with the purpose and scope of the ESGO consensus statements. Subsequently, the first drafts of evidence-based consensus statements were written using the available evidence. An adapted version of the Infectious Diseases Society of America–US Public Health Service Grading System was used to define the LOE and grade of recommendation for each statement^{8,9} (eTable in the Supplement).

During a second conference call, the group discussed each preliminary statement. Anonymous voting followed online via SurveyMonkey, which was binary (agree/disagree) for each potential statement. Participants were permitted to abstain from voting if they thought they had insufficient expertise to agree or disagree with the statement or if they had a conflict of interest that could be considered to influence their vote. No participant abstained from voting due to any potential conflict of interest. Participants could provide comments and suggestions with their votes. The voting results and revised consensus statement were

Figure. Eight-Step Process for Development of the Consensus Statements on Opportunistic Salpingectomy for Prevention of Tubo-Ovarian Carcinoma**1 Nomination of a multidisciplinary international group**

- Members of the European Society of Gynaecological Oncology (ESGO) Prevention Committee were nominated and subsequently selected for the project by the ESGO Prevention Committee Executive Council, based on subject matter expertise, number of publications, and previous experience with consensus statements
- The selection included gynecologists and gynecological oncologists from 9 different countries, and 1 patient representative

Identification of the clinical questions

An initial call was conducted to describe the clinical issues to be addressed and define the 10 PICOs

- Incidence of tubo-ovarian carcinoma (PICOs 1-2)
- Incidence of premature ovarian insufficiency or failure (PICOs 3-5)
- Short-term and long-term health risks (PICOs 6-8)
- Postoperative complications (PICOs 9-10)

2 Identification of the scientific evidence

- Search designed and conducted by a methodologist in MEDLINE
- January 1, 2000–March 1, 2025; limited to English language
- Search results distributed to all members of the full group, who were given time to read and review included studies
- Development of 4 working groups; each working group screened the evidence for each PICO separately and drafted individual statements

Grading of the evidence

An adapted version of the Infectious Diseases Society of America–US Public Health Service Grading System was used to define the level of evidence and grade for each statement

3 Formation of each preliminary consensus statement

- Modified Delphi approach
- Preliminary statements were disseminated to all members of the study group, who were asked to review individually ahead of an online meeting

4 Discussion of each preliminary consensus statement

- During an online meeting, the statements were discussed by all members of the study group and edited
- Opportunity for nonanonymous and anonymous feedback online after the call
- Feedback on statements collected and statements reedited

5 First round of voting

- First statements drafted and distributed for first round of voting (online)
- Modified Delphi; collaborators requested to vote anonymously on each recommendation (voting options were agree, disagree, or abstain)
- Mandatory comments and reasons for decision (if disagreed or abstained)

6 Revision of statements

- Statements amended on an online conference call based on anonymous group feedback

7 Second round of voting

- Recommendations with 100% agreement accepted, and recommendations with <100% agreement underwent voting in round 2
- Statements and recommendations with >75% agreement were accepted (final draft)
- Language of recommendations was refined with consideration of contributors' comments

8 Finalization of statements

- Final statements with a high rate of agreement produced (>75%)

Manuscript preparation

The manuscript was drafted by the lead author with input and editing by all authors

PICO indicates patient or problem; intervention or exposure; comparison or control; outcome(s).

provided to the group. A second round of binary anonymous voting (SurveyMonkey) was conducted for only the revised statements according to the same procedure to allow the full group to evaluate the revised statements. The finalized consensus statements were based on the results of the second round of voting. The finalized evidence-based consensus statements, together

with a summary of evidence supporting each statement, are detailed in this article.

Responsibilities

Our aim is to present evidence-based consensus statements on opportunistic salpingectomy for prevention of tubo-ovarian

carcinoma; however, ESGO and the international development group acknowledge that there will be broad geographical variability in practice, with significant differences in resources, infrastructure, and technology. Moreover, variation in training, medicolegal, financial, and cultural factors may affect the implementation and applicability of any consensus statement in each country and health care system. These consensus statements are a statement of evidence and consensus of the development group regarding their views and perspective of currently accepted approaches for opportunistic salpingectomy for prevention of tubo-ovarian carcinoma. Any clinician applying or consulting these statements is expected to use independent medical judgment in the context of individual clinical circumstances to determine any patient's management.

Measures

Ten distinct PICO frameworks were used to explore a broad range of comparisons concerning tubo-ovarian carcinoma risk, perioperative concerns, ovarian function, and long-term health risks associated with removal of fallopian tubes (see the PICOs listed in the eAppendix in the [Supplement](#)). As a bilateral salpingectomy is clearly a sterilization procedure, the appropriate population includes only those with no wish to retain fertility. The guidance and statements in this document do not apply to women wishing to retain fertility.

The primary outcomes assessed included

- Incidence of tubo-ovarian carcinoma (PICOs 1-2)
- Incidence of premature ovarian insufficiency or failure (PICOs 3-5)
- Short- and long-term health risks (PICOs 6-8)
- Postoperative complications (PICOs 9-10)

Data Sources

Data were derived from searches in PubMed (MEDLINE). For effect on tubo-ovarian carcinoma incidence (PICOs 1-2), data were extracted from women with and without salpingectomy (for any reason) in their history. For ovarian function outcomes (PICOs 3-5), hormone data (follicle-stimulating hormone [FSH], antimüllerian hormone [AMH], estradiol), age at menopause, and rates of hormone therapy (in all formulations, systemic or vaginal only) were extracted. For health risk and complication analyses (PICOs 6-10), metrics included all-cause mortality, cardiovascular disease, osteoporosis, neurocognitive decline, mental health diagnoses, and adverse surgical outcomes including complications.

Analytic Methods

To evaluate trends in tubo-ovarian carcinoma incidence after opportunistic salpingectomy (PICOs 1-2), cumulative incidence and adjusted odds ratios or hazard ratios were obtained. For ovarian reserve (PICOs 3-5), time to menopause and biochemical markers were analyzed. Health risks and complications (PICOs 6-10) were analyzed based on available longitudinal cohort data.

Results

A total of 230 studies were short-listed for review by the methodologist. After review by the working groups, 129 articles were selected to formulate statements (see the eReferences in the [Supplement](#)). Twenty-one of these studies and their corresponding LOEs (eTable in the [Supplement](#)) are discussed in the following section.

Summary of the Evidence and Consensus Statements

Tubo-Ovarian Carcinoma Risk Reduction

Multiple systematic reviews and meta-analyses demonstrate that salpingectomy is associated with a substantial reduction in the risk of tubo-ovarian carcinoma, although much of the data are retrospective and there are no RCTs. The largest systematic review was published in 2023 by Kahn et al,¹⁰ which included 158 studies with various study designs and reported that bilateral salpingectomy could reduce tubo-ovarian carcinoma risk by up to 80% compared with no salpingectomy and could lower ovarian cancer mortality in the United States by 15% if broadly implemented, based on decision-analysis modeling and cohort data (LOE IV). A systematic review and meta-analysis published in 2025 by Tang et al¹¹ including 1 cohort study and 4 case-control studies found that the risk of developing tubo-ovarian carcinoma among women who underwent bilateral salpingectomy ($n = 5455$) was significantly lower compared with those who did not undergo the procedure ($n = 5721606$) (odds ratio, 0.48; 95% CI, 0.33-0.69; $P < .001$) (LOE IV). Almost a decade earlier, Yoon et al¹² published a meta-analysis, 2 of the 3 studies from which were also included in the analysis by Kahn and colleagues.¹⁰ This 2016 meta-analysis featured 1 cohort study and 2 population-based case-control studies and similarly found a significant reduction in tubo-ovarian carcinoma risk in women who underwent bilateral salpingectomy ($n = 3509$) compared with those who did not undergo salpingectomy ($n = 5655702$) (odds ratio, 0.51; 95% CI, 0.35-0.75; $P < .001$) (LOE IV).¹² The study contributing the most patients to both of these meta-analyses^{11,12} was a nationwide population-based cohort study in Sweden by Falconer et al,¹³ which analyzed data on women with previous surgery for a benign gynecological indication, of which 3051 underwent bilateral salpingectomy compared with an unexposed population ($n = 5449119$), between 1973 and 2009. This study reported a significantly lower risk of tubo-ovarian carcinoma in women undergoing bilateral salpingectomy compared with those who did not undergo salpingectomy (incidence rate calculated per 100 000 person-years, 9.9 vs 25.3, respectively; hazard ratio, 0.35; 95% CI, 0.17-0.73; $P = .04$) (LOE IV).¹³ The second study featured in both reviews^{11,12} was a nationwide register-based case-control study conducted between 1982 and 2011 in Denmark by Madsen et al,¹⁴ including all women between 30 and 84 years of age with a first diagnosis of histologically confirmed epithelial ovarian cancer or borderline ovarian tumors. The authors reported a significant reduction in tubo-ovarian carcinoma risk in women who underwent bilateral salpingectomy compared with those who did not (odds ratio, 0.58; 95% CI, 0.36-0.95) (LOE IV). However, this analysis was based on a few exposed cases who underwent bilateral salpingectomy ($n = 17$) and a control group who did not undergo salpingectomy ($n = 411$).¹⁴

Ovarian Function and Premature Ovarian Failure

Evidence from RCTs and systematic reviews indicates that opportunistic salpingectomy does not adversely affect short-term ovarian reserve. In a small RCT by Asgari et al¹⁵ conducted in Iran between 2013 and 2015, including 15 patients who underwent hysterectomy with bilateral salpingectomy and 15 patients who underwent hysterectomy without salpingectomy, no significant changes in AMH or FSH levels 3 months after laparoscopic hysterectomy with or without opportunistic salpingectomy were observed. The mean postoperative AMH level was 1.13 (SD, 0.86) ng/dL

in the salpingectomy group vs 0.97 (SD, 0.92) ng/dL in the group who did not undergo salpingectomy ($P = .62$), and the mean postoperative FSH level was 8.46 (SD, 2.18) mIU/mL in the salpingectomy group compared with 8.8 (SD, 1.92) mIU/mL in the group who did not undergo salpingectomy ($P = .65$) (LOE II).¹⁵ Similarly, a small pilot RCT conducted in the United States by Findley et al,¹⁶ including 15 patients who underwent hysterectomy with bilateral salpingectomy and 15 patients who underwent hysterectomy alone, found no significant difference in AMH levels 3 months postoperatively between the 2 groups (mean AMH change, -0.07 [SD, 0.90] ng/mL vs -0.08 [SD, 1.45] ng/mL, respectively; $P = .98$) (LOE II). A 2017 pooled analysis by Mohamed et al¹⁷ included the small RCT by Findley et al,¹⁶ 1 additional RCT, 3 prospective cohort studies, and 1 retrospective cohort study. These authors found no statistically significant reduction in AMH levels following bilateral salpingectomy ($n = 383$) compared with AMH levels before salpingectomy ($n = 464$) (weighted mean difference, -0.10 ng/mL; 95% CI, -0.19 to 0.00 ng/mL; $I^2 = 0\%$) (LOE III).¹⁷ However, observational studies provide mixed results on menopause-related outcomes. A registry-based cohort study in Sweden by Collins et al¹⁸ included women younger than 55 years who had a hysterectomy for benign indications with or without bilateral salpingectomy between 1998 and 2016. A significant increased risk of menopausal symptoms was reported, as registered by the patients in questionnaires, 1 year after hysterectomy with opportunistic salpingectomy ($n = 6892$) compared with hysterectomy alone ($n = 4906$) (relative risk, 1.33; 95% CI, 1.04-1.69) (LOE IV).¹⁸ In contrast, a cohort study in British Columbia by Hanley et al,¹⁹ which identified all women aged 50 years or younger who underwent opportunistic salpingectomy at hysterectomy from 2008 through 2014 ($n = 6500$), reported no significant differences in time to menopause-related physician visits compared with women who underwent hysterectomy alone ($n = 6861$) (adjusted hazard ratio, 0.98; 95% CI, 0.88-1.09) (LOE IV). Additionally, this study reported that time to filling a prescription for hormone therapy for women who underwent hysterectomy with opportunistic salpingectomy or opportunistic salpingectomy for sterilization was similar to that among women who underwent hysterectomy alone or sterilization by tubal ligation (adjusted hazard ratios, 0.82 [95% CI, 0.72-0.92] and 1.00 [95% CI, 0.89-1.12], respectively) (LOE IV).¹⁹

Surgical Outcomes and Complications of Opportunistic Salpingectomy During Gynecological Surgeries

Surgical outcomes from RCTs and large cohort studies indicate that opportunistic salpingectomy adds minimal operative time during cesarean delivery, abdominal hysterectomy, and laparoscopic hysterectomy and does not increase perioperative risks. A small RCT by Subramaniam et al,²⁰ performed between 2015 and 2017 in the United Kingdom, included 40 patients who underwent opportunistic salpingectomy and 40 patients who underwent bilateral tubal ligation during cesarean delivery and reported that opportunistic salpingectomy added a mean of 15 minutes compared with tubal ligation (75.4 [SD, 29.1] minutes compared with 60.0 [SD, 23.3] minutes; $P = .004$), with no adverse outcomes directly related to the sterilization procedures in both groups (LOE II). Another small RCT ($n = 68$) by Song et al,²¹ conducted between

2013 and 2015 in Korea, found no significant differences in operative time, blood loss, or complications between the opportunistic salpingectomy ($n = 34$) and nonopportunistic salpingectomy ($n = 34$) groups at laparoscopic hysterectomy (operative time: 95 [range, 69-110] minutes vs 90 [range, 65-106] minutes; $P = .55$; blood loss: 125 mL [range, 75-200 mL] vs 150 mL [range, 100-225 mL]; $P = .36$; complications: 0 vs 2 postoperative complications; $P = .49$) (LOE II). Similarly, 2 other small RCTs by Asgari et al ($n = 30$)¹⁵ and by Findley et al ($n = 30$)¹⁶ reported no differences in operative time with the addition of opportunistic salpingectomy to laparoscopic hysterectomy compared with laparoscopic hysterectomy alone (mean operative times, 126.33 [SD, 28.43] minutes vs 126.66 [SD, 31.77] minutes [$P = .97$] and 115.7 [SD, 33] minutes vs 115.2 [SD, 44] minutes [$P = .97$], respectively). Additionally, these 2 RCTs reported no differences in blood loss (mean blood loss, 49.33 mL [SD, 22.82 mL] vs 48.66 mL [SD, 17.26 mL] [$P = .92$] and 70.3 mL [SD, 50 mL] vs 91.3 mL [SD, 121 mL] [$P = .54$], respectively) (LOE II).^{15,16} A systematic review and meta-analysis by Bonavina et al²² published in 2024 included 7 observational prospective and retrospective cohort studies. This study found that successful opportunistic salpingectomy during vaginal hysterectomy was not associated with increased postoperative complications compared with vaginal hysterectomy and unsuccessful salpingectomy (odds ratio, 0.45; 95% CI, 0.19-1.03; $P = .06$), and no increases in estimated blood loss (mean difference, -0.11 mL; 95% CI, -0.40 to 0.18 mL; $P = .46$) or length of hospital stay (mean difference, -0.02 days; 95% CI, -0.25 to 0.20 days; $P = .85$) were observed (LOE IV).²² The Cochrane review by van Lieshout et al²³ published in 2019 ($n = 286$; 5 small RCTs, including Findley et al¹⁶ and Song et al²¹) concluded that opportunistic salpingectomy at the time of hysterectomy was not associated with a higher risk of intraoperative complications compared with hysterectomy alone (odds ratio, 0.66; 95% CI, 0.11-3.94; $P = .65$). In addition, this Cochrane review concluded that opportunistic salpingectomy at the time of hysterectomy was not associated with a higher risk of short-term postoperative complications compared with hysterectomy alone ($n = 152$; 3 small RCTs; odds ratio, 0.13; 95% CI, 0.01-2.14; $P = .15$), although the quality of evidence was very low (LOE II).²³

Feasibility of Opportunistic Salpingectomy During Nongynecological Surgeries

A feasibility study conducted in Austria by Tomasch et al²⁴ demonstrated that opportunistic salpingectomy is feasible during laparoscopic cholecystectomy. Opportunistic salpingectomy was successfully completed in 93.3% (98/105) of included patients, while the procedure was unable to be completed in 6.7% (7/105) due to adhesions. The median additional operative time was 13 minutes (range, 4-45 minutes), and no complications were attributed to opportunistic salpingectomy. In this study, opportunistic salpingectomy was carried out by general surgeons without input from gynecologists in the majority of cases (75/105 [75.3%]) (LOE IV). In a small preliminary study by Williams (abstract only)²⁵ conducted between 2011 and 2018, bilateral salpingectomy was performed in 19 patients undergoing bariatric surgery, of which 1 (5.3%) was not feasible due to adhesions. The mean extraoperative time was 8.1 minutes, and there were no additional perioperative complications (LOE IV). In addition, visualization of the fallopian tubes has also

been shown to be feasible during bariatric surgery (LOE IV).²⁶ A clinical trial registered in Canada (NCT05300711) will investigate the feasibility of opportunistic salpingectomy during colorectal surgery. Additionally, several cost-effectiveness analyses have evaluated opportunistic salpingectomy performed during nongynecological surgeries (LOE IV).^{27,28} These studies indicate that opportunistic salpingectomy is a cost-effective and, depending on the setting, cost-saving strategy in benign pelvic and selected abdominal surgeries. The nongynecological surgeries included in these analyses were cholecystectomy, hernia repair, bariatric surgery, colon resection, and planned uncomplicated appendectomy. However, these studies were based on modeled populations within a Markov model, state transition model, or decision model, respectively, and therefore the results should be interpreted with caution. A scoping review by Fisch et al²⁹ published in 2024 described 18 studies and reported on implementation, feasibility, patient perspectives, and cost-effectiveness. This review concluded that opportunistic salpingectomy performed during nongynecological pelvic and selected abdominal surgeries is feasible and promising; however, it lacks well-defined implementation strategies regarding surgical training and the resolution of organizational challenges (LOE IV). The quality of the 18 included studies was mixed (9 conference abstracts, 4 ongoing trials, and 5 full-text articles).²⁹

Statement From the Expert Group Based on the Evidence

Based on current evidence and expert consensus, opportunistic salpingectomy should be considered for all eligible women undergoing gynecological surgery and, when possible, nongynecological pelvic or selected abdominal surgeries as a preventive measure against tubo-ovarian carcinoma. Counseling should include that opportunistic salpingectomy does not impair short-term ovarian function and does not appear to affect age at menopause or menopausal symptoms. However, long-term data remain limited, and a possible impact on ovarian function or earlier onset of menopause cannot be fully excluded. Among eligible women, age alone should not be used to withhold opportunistic salpingectomy, although data are insufficient to ascertain which age group is likely to benefit the most. While certain surgical risk factors (eg, severe adhesions) may limit feasibility, opportunistic salpingectomy is generally feasible in most pelvic and selected abdominal surgeries. Patients do not require specific additional follow-up after opportunistic salpingectomy beyond standard postoperative care. These statements emphasize individualized counseling to support shared decision-making regarding opportunistic salpingectomy for tubo-ovarian carcinoma prevention (Table). Currently, high-quality evidence is lacking, particularly regarding conducting opportunistic salpingectomy during nongynecological procedures, the true effect on menopausal symptoms, and the impact of conducting this procedure at extremes of age. We anticipate future research aimed at providing high-quality data to answer these knowledge gaps.

Discussion

This literature review and expert consensus process supports the increasing body of evidence that opportunistic salpingectomy is associated with a significant risk-reducing benefit against tubo-

ovarian carcinoma in the general population. Multiple observational studies and meta-analyses indicate a consistent association between opportunistic salpingectomy and lower tubo-ovarian carcinoma incidence, with risk reductions ranging from 42% to 80%.^{13,14,30-32} These findings reinforce the plausibility of the fallopian tube as the primary site of origin for high-grade serous ovarian cancers and validate opportunistic salpingectomy as a viable preventive strategy. No evidence was found that opportunistic salpingectomy influences the risk of other histological types of ovarian cancer.

Importantly, the data also suggest that opportunistic salpingectomy does not significantly compromise short-term ovarian function. Across RCTs and systematic reviews, hormone markers such as AMH and FSH remained stable postoperatively, and no substantial differences in ovarian reserve were observed.⁴⁻³⁷ However, findings from observational studies on long-term menopause-related outcomes are inconsistent. While some studies indicated increased risk of menopausal symptoms following opportunistic salpingectomy,^{19,38} others found no effect on the timing of menopause or the need for hormone therapy. This highlights a persistent knowledge gap regarding the long-term endocrine consequences of opportunistic salpingectomy, particularly in younger women. Therefore, premenopausal women should be counseled accordingly.

In terms of perioperative safety, evidence indicates that opportunistic salpingectomy is a technically feasible procedure with minimal added operative time and no significant increase in surgical complications.^{15,16,20,33,36,39,40} These findings are consistent for both gynecological and nongynecological procedures and approaches, such as laparoscopic and open surgeries. The evidence suggests that a vaginal approach may increase risk of complications compared with abdominal approaches, which should be considered in preoperative planning and counseling of patients. Altogether, this underscores the practicality of incorporating opportunistic salpingectomy into routine gynecological surgical practice without a substantial burden of additional risk or resource utilization.

Despite these promising data, implementation of opportunistic salpingectomy remains inconsistent, reflecting ongoing debate about long-term safety, feasibility across diverse clinical settings, and patient counseling strategies. The lack of RCTs assessing long-term non-cancer-related outcomes and the limited data in subpopulations, such as women younger than 35 years, contribute to this uncertainty. Furthermore, the absence of subgroup analyses precludes definitive conclusions regarding age-specific effects. The current consensus acknowledges these gaps and advocates for transparent patient counseling, including discussion of the potential risks to ovarian function.

An important consideration for future research is the existing lack of evidence regarding postoperative regret in relation to fertility following opportunistic salpingectomy. Regret has primarily been reported in studies on sterilization, particularly among younger women.⁴¹ Although this has not yet been investigated thoroughly in the context of opportunistic salpingectomy, the potential for regret warrants attention. Currently, many international societies including the International Federation of Gynecology and Obstetrics (FIGO) recommend salpingectomy as the preferred method of sterilization for women, rather than tubal ligation, given its potential to reduce the risk of tubo-ovarian carcinoma⁴²⁻⁴⁵; however, there are

Table. List of the European Society of Gynaecological Oncology Evidence-Based Consensus Statements

Statement	Consensus, % (No./total)	Grade ^a	Level of evidence ^b
1. All women eligible should be counseled on the option of opportunistic salpingectomy.	100 (12/12)	B	IV
2. Opportunistic salpingectomy should be considered for all eligible women undergoing gynecological pelvic surgery as a preventive measure against tubo-ovarian carcinoma.	100 (12/12)	B	IV
3. Opportunistic salpingectomy should be considered for all eligible women undergoing nongynecological pelvic or selected abdominal surgery as a preventive measure against tubo-ovarian carcinoma.	100 (12/12)	B	V
4. Clinicians should inform women who will undergo pelvic or selected abdominal surgery with ovarian preservation that adding a salpingectomy does not impair short-term ovarian function.	100 (12/12)	B	II
5. Clinicians should inform women who will undergo pelvic or selected abdominal surgery with ovarian preservation that opportunistic salpingectomy most likely does not impact ovarian reserve; however, long-term evidence is lacking.	100 (12/12)	B	IV
6. Clinicians should inform women who will undergo opportunistic salpingectomy with ovarian preservation that there are only limited evidence-based data on long-term effects on ovarian function.	91.67 (11/12)	B	IV
7. Clinicians should inform women who will undergo pelvic or selected abdominal surgery with ovarian preservation that opportunistic salpingectomy does not seem to have an impact on age at menopause or menopausal symptomatology.	100 (12/12)	B	IV
8. Due to absence of evidence, age alone cannot be used to withhold opportunistic salpingectomy. Premenopausal women need to be fully counseled as part of the surgical consent process and informed about the resultant sterilization and loss of fertility associated with the procedure. Consent should not be obtained as part of an emergency procedure or without adequate prior time for consideration.	100 (12/12)	C	V
9. Women need to be counseled on the possible effect on age at menopause onset/possible risk of ovarian degeneration following opportunistic salpingectomy.	83.33 (10/12)	B	IV
10. Enrollment of patients undergoing opportunistic salpingectomy into prospective studies is encouraged.	100 (12/12)	B	V
11. Patients undergoing opportunistic salpingectomy do not require specific surgical follow-up.	100 (12/12)	D	IV
12. Although certain risk factors for feasibility of opportunistic salpingectomy during planned surgery for benign indications are identified in the literature, it remains a feasible procedure for many patients undergoing different surgical approaches, for both gynecological and nongynecological pelvic or selected abdominal indications.	100 (12/12)	B	III
13. If there are massive adhesions or other risk factors for surgical complications, one should not proceed with opportunistic salpingectomy.	100 (12/12)	D	III
14. Due to the absence of targeted subgroup analyses, the characteristics of identified short- and long-term health risks of opportunistic salpingectomy in women older than 65 years are insufficient.	100 (12/12)	C	V
15. Clinicians should counsel patients that performing an opportunistic salpingectomy (either as an additional procedure or in place of bilateral tubal ligation) at the time of cesarean delivery adds less than 15 minutes of operative time in comparison with tubal ligation.	100 (12/12)	B	II
16. Clinicians should counsel patients that performing an opportunistic salpingectomy at the time of cesarean delivery does not result in a significantly increased risk of complications compared with tubal ligation.	100 (12/12)	B	II
17. Clinicians should counsel patients that performing opportunistic salpingectomy at the time of abdominal hysterectomy (minimally invasive or laparotomy) does not result in a significantly increased risk of complications.	100 (12/12)	B	II
18. Clinicians should counsel patients that performing an opportunistic salpingectomy at the time of vaginal hysterectomy may result in an increased risk of surgical complications; however, overall, the risk of a major complication remains low.	91.67 (11/12)	B	IV

^a Grades of recommendation: grade B, strong or moderate evidence for efficacy but with a limited clinical benefit, generally recommended; grade C, insufficient evidence for efficacy or benefit does not outweigh the risk or disadvantages (adverse events, costs, etc), optional; and grade D: moderate evidence against efficacy or for adverse outcome, generally not recommended.

^b Levels of evidence: II, small or large randomized clinical trials with a suspicion of bias (lower methodological quality) or meta-analyses of such trials or of trials with demonstrated heterogeneity; III: prospective cohort studies; IV: retrospective cohort studies or case-control studies; and V: studies without a control group, case reports, and expert opinions.

no recommendations specific to premenopausal younger women. Clearly, an individualized approach should be taken to opportunistic salpingectomy, which includes adequate counseling and time for prior consideration by patients. Opportunistic salpingectomy should not be performed as part of an emergency surgery or in any circumstances in which future fertility desires are not known.

The strengths of this review include a rigorous literature synthesis using clearly defined PICO frameworks, stratification by key clinical variables, and the engagement of an expert panel using a structured, iterative voting process.

Future Directions Regarding Opportunistic Salpingectomy During Select Nongynecological Abdominal Surgeries

Small case series have demonstrated that opportunistic salpingectomy is feasible during nongynecological pelvic and selected abdominal surgeries.^{24,25} As a result, opportunities to implement opportunistic salpingectomy in such surgical contexts should be considered. However, implementation would be optimized if a successful implementation strategy framework was available either at a funding or organization level regarding opportunistic salpingectomy during nongynecological pelvic and selected abdominal

procedures.⁴⁶ Potential practical strategies to aid implementation include specific accreditation for general surgeons, cross-specialty operative planning, or an on-call gynecologist designated for this purpose. The exact strategy will need to be adapted to each relevant health care setting, but international and national societies should play a role in issuing guidance. Several clinical trials are registered that will continue to investigate this topic (NCT05300711, NCT06312124). These larger-scale studies are expected to clarify when opportunistic salpingectomy is feasible and which implementation strategies are most effective.

Implications for Clinical Practice and Research

Given the available evidence, opportunistic salpingectomy should be considered in all gynecological surgeries, and when possible, for elective nongynecological pelvic and selected abdominal surgeries, in women who do not desire future fertility, are at average population risk of tubo-ovarian carcinoma, and are not undergoing oophorectomy. Clinicians should provide clear, balanced information about the cancer risk reduction benefits and the currently limited evidence on long-term ovarian health. Age alone should not be used to exclude patients from the potential benefits of opportunistic salpingectomy, although counseling should reflect the current uncertainty regarding its impact on the timing of menopause. Future research should prioritize long-term prospective studies with standardized hormone assessments and follow-up, especially in younger and older age strata.

Limitations

This report has several limitations. First, the reliance on primarily observational data for cancer and long-term hormone outcomes involves inherent risks of confounding and selection bias. Second, the generalizability of findings to specific populations, including postmenopausal women, women younger than 35 years, and those undergoing nongynecological surgeries, remains inadequately addressed in the current literature.

Third, the consensus statements in this article were developed without a specific economic analysis of the strategies. Fourth, many of the included studies were conducted in high- and medium-income settings, and thus the findings may or may not be generalizable to low-income settings.

Conclusions

Currently available evidence shows that opportunistic salpingectomy is significantly associated with a lower risk of developing tubo-ovarian carcinoma, does not adversely affect short-term ovarian reserve, adds minimal operative time, and does not increase perioperative risks. Therefore, opportunistic salpingectomy should be considered in all gynecological surgeries, and when possible, for elective nongynecological pelvic and selected abdominal surgeries, in women who are at general risk of developing tubo-ovarian carcinoma and do not desire future fertility.

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Author Affiliations: Department of Obstetrics and Gynecology, Catharina Cancer Institute, Catharina Hospital, Eindhoven, the Netherlands (Piek, Schauwaert); Department of Obstetrics and Gynecology, Radboud University Medical Center, Nijmegen, the Netherlands (Schauwaert, Steenbeek); Department of Metabolism, Digestion and Reproduction—Surgery and Cancer, Imperial College London, London, United Kingdom (Ellis, Bowden, Kyrgiou); Imperial College Healthcare NHS Trust, London, United Kingdom (Ellis, Bowden, Kyrgiou); Department of Gynecologic Oncology, La Paz University Hospital, Madrid, Spain (Zapardiel); Bergonié Institute, Bordeaux, France (Planchamp); Mallow Flower Foundation, Budapest, Hungary (Koblos); Department of Gynecological Oncology, Maria Skłodowska-Curie National Research Institute of Oncology, Warsaw, Poland (Kacperczyk-Bartnik); Department of Obstetrics and Gynecology, Medical University of Warsaw, Warsaw, Poland (Kacperczyk-Bartnik); Department of Gynecological Oncology, Gustave Roussy Institute, Villejuif, France (El Hajj); University of Medicine and Pharmacy Grigore T. Popa, Iasi, Romania (Grigore); UOC Ginecologia Oncologica, Dipartimento di Scienze della Salute della Donna, del Bambino e di Sanità Pubblica, Fondazione Policlinico Universitario A. Gemelli IRCCS, Rome, Italy (Bizzarri); Department of Obstetrics and Gynecology, Hacettepe University Faculty of Medicine, Ankara, Türkiye (Gültekin).

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